RAHATI

Women's Mental Well-being: Survey Findings and Preliminary Research Review

Detailed report based on a questionnaire of 40 respondents

Prepared for the Rahati project



Executive Summary

This report presents the findings of a questionnaire conducted with 40 respondents to better understand women's mental well-being challenges, their priorities, and the features they would expect from Rahati. It also summarizes the project's preliminary review of research and available data related to women's mental health and the menstrual cycle.

The survey points to three recurring needs: easier access to trusted support, better understanding of emotions and mood changes, and practical tools for managing stress and everyday well-being. Respondents also expressed strong expectations regarding privacy, confidentiality, affordability, and the possibility of remaining anonymous.

The preliminary research review suggests that research specifically examining women's mental well-being in relation to menstrual-cycle phases, particularly among African and Arab women, is limited in the sources identified by the project team. The mcPHASES dataset was identified as a relevant resource because it combines menstrual-cycle information, hormonal measurements, and self-reported symptoms, including mood swings. However, the variables available in this dataset do not cover the full range of psychological, emotional, and social factors needed to understand women's mental health comprehensively.

These findings inform Rahati's direction: a women-centered platform combining self-monitoring, educational resources, practical self-care tools, progress summaries, and optional access to trusted professionals.

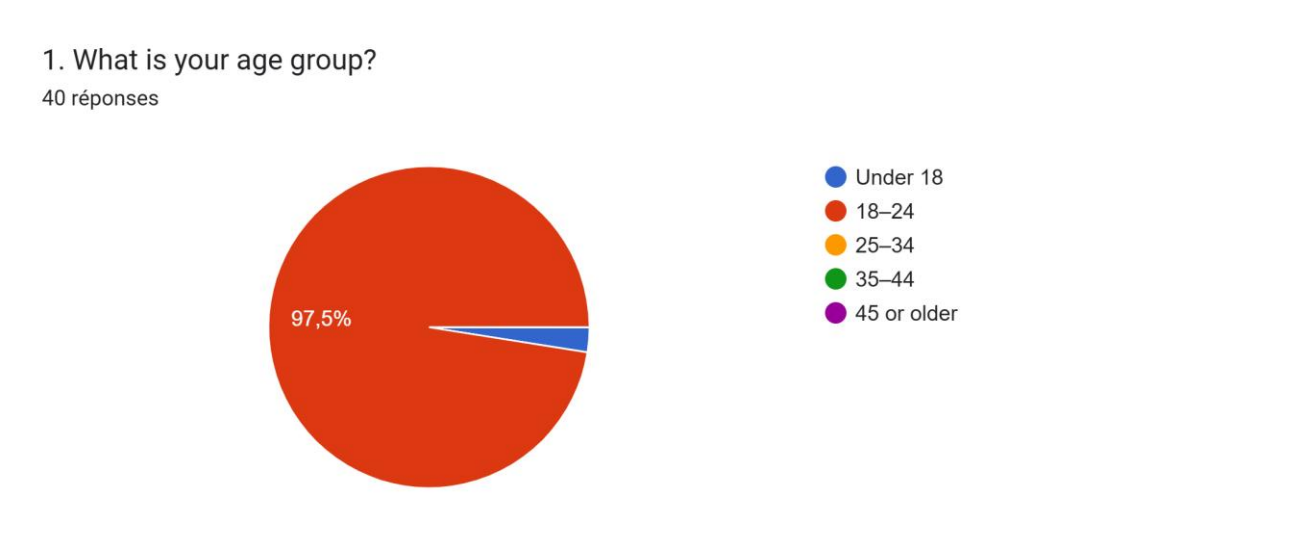
1. Survey Background and Methodology

1.1 Purpose of the questionnaire

The questionnaire was designed to explore the barriers women face when taking care of their mental well-being, what they would like to understand or improve, which Rahati features they consider useful, and what would encourage them to use a digital mental well-being platform.

1.2 Respondent profile

The questionnaire received 40 responses. According to the age distribution provided, 97.5% of respondents were between 18 and 24 years old, while the remaining 2.5% were under 18.



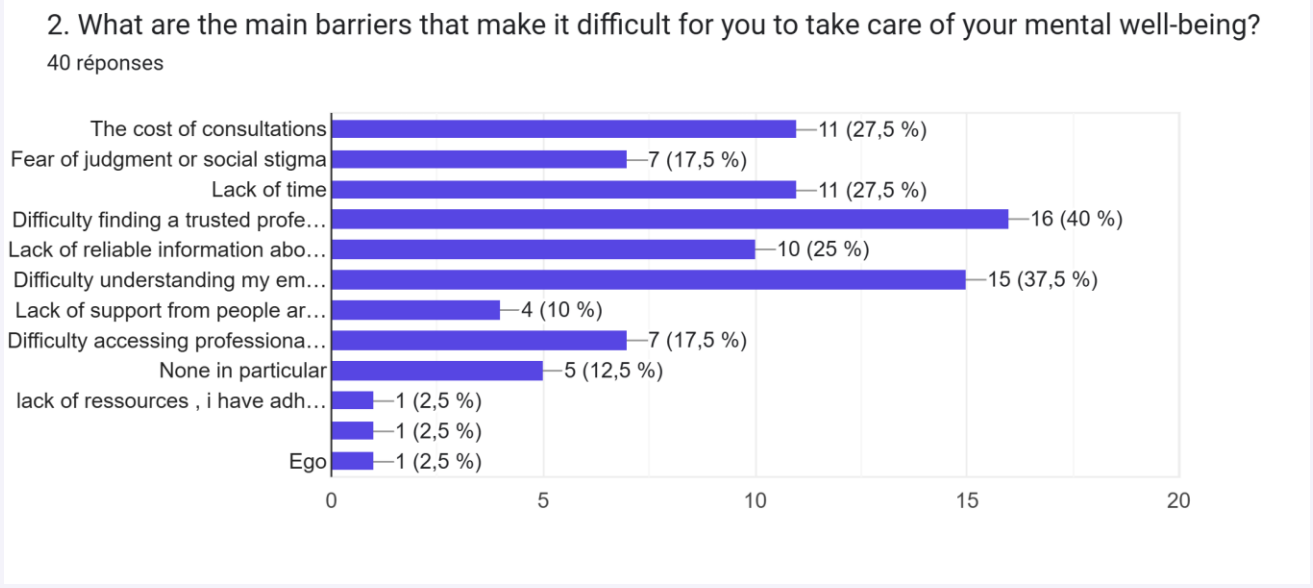
The age profile indicates that the survey mainly reflects the views of young adult women. The findings should therefore not be interpreted as representative of women of all age groups.

1.3 How to interpret the results

Most questions shown in the survey allowed respondents to select more than one answer. Consequently, percentages represent the proportion of respondents selecting each option and should not be added together. Unless otherwise indicated, the percentages below are based on 40 respondents. Small differences may result from rounding.

2. Main Barriers to Women's Mental Well-being

Respondents were asked: "What are the main barriers that make it difficult for you to take care of your mental well-being?" The most frequently selected barrier was difficulty finding a trusted professional, selected by 16 respondents (40%). This was followed by difficulty understanding personal emotions, selected by 15 respondents (37.5%).



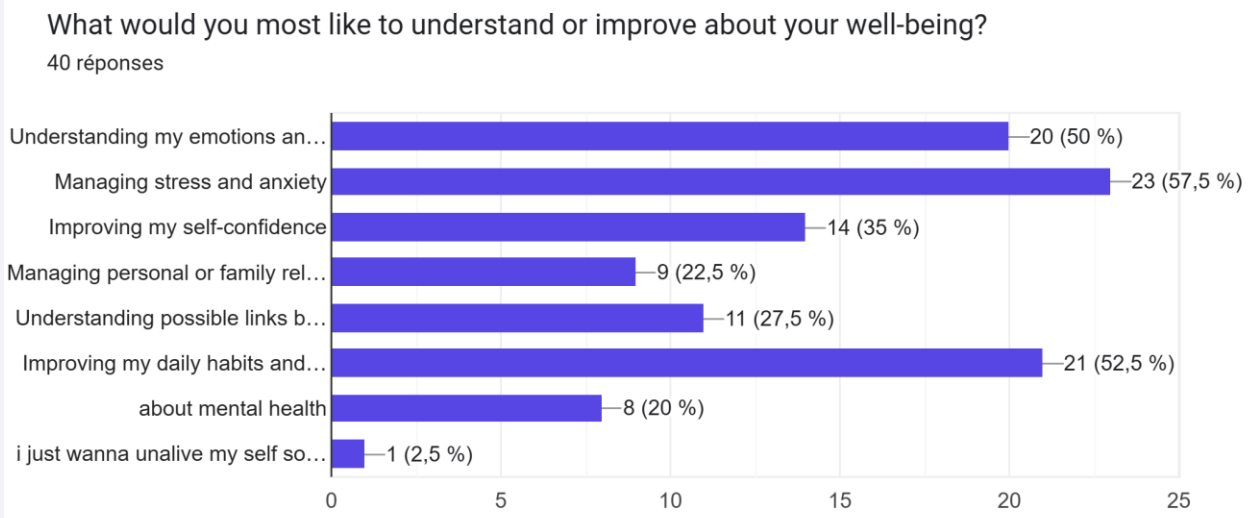
2.1 Interpretation

The results suggest that barriers are not limited to the availability of mental health services. A substantial proportion of respondents also report difficulty understanding their own emotions. This points to a need for accessible tools that help users reflect on their emotional state and recognize changes over time.

The cost of consultations and lack of time were each selected by 27.5% of respondents. These findings support the importance of offering flexible and affordable resources. In addition, one quarter of respondents reported a lack of reliable mental health information, indicating a potential role for clear, accessible, and trustworthy educational content.

3. What Women Want to Understand or Improve

Respondents were asked what they would most like to understand or improve about their well-being. Managing stress and anxiety was the most frequently selected priority, with 23 responses (57.5%). Improving daily habits and sleep followed closely, with 21 responses (52.5%). Understanding emotions and mood changes was selected by 20 respondents (50%).



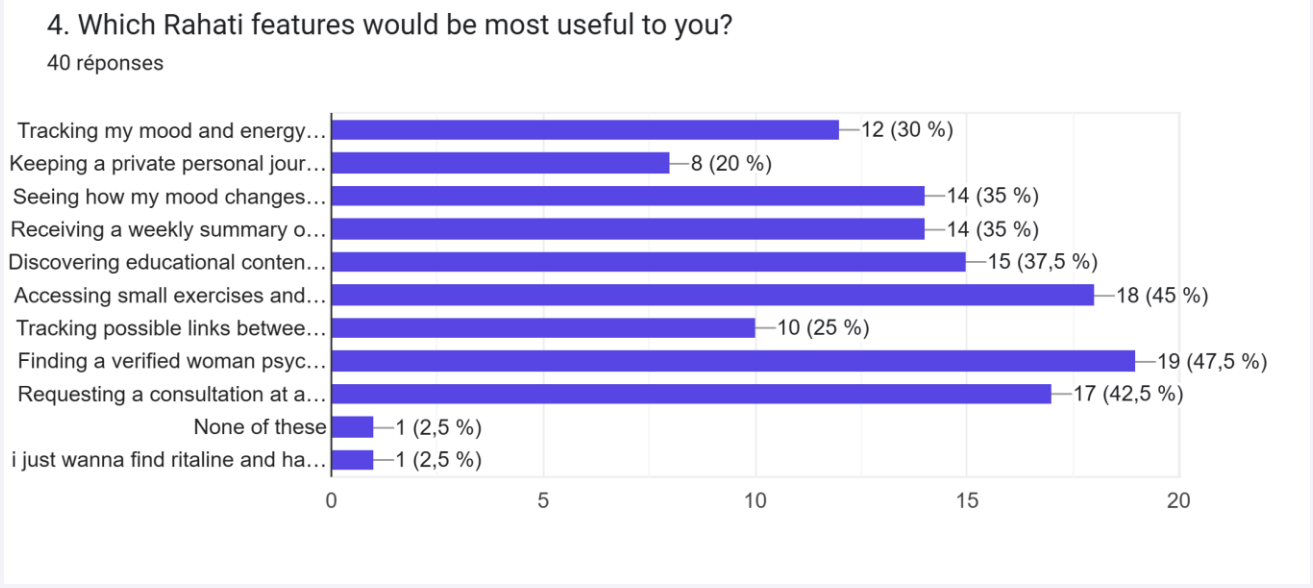
3.1 Interpretation

The responses show that participants are looking for practical support in their everyday lives. Stress management, sleep, daily habits, and emotional self-understanding are prominent priorities. Half of respondents specifically selected understanding emotions and mood changes.

A possible relationship between the menstrual cycle and emotions was selected by 27.5% of respondents. Although this is not the highest-ranked priority, it provides a clear indication that cycle-related emotional patterns are relevant to a segment of the target audience. Rahati can make cycle tracking an optional feature rather than assuming that every user wants or needs it.

4. Features Expected from Rahati

Respondents were asked which Rahati features would be most useful to them. The most frequently selected feature was finding a verified woman psychologist or therapist, selected by 19 respondents (47.5%). Access to small exercises and personal goals followed, with 18 responses (45%). Requesting a consultation at an affordable price was selected by 17 respondents (42.5%).



4.1 Interpretation

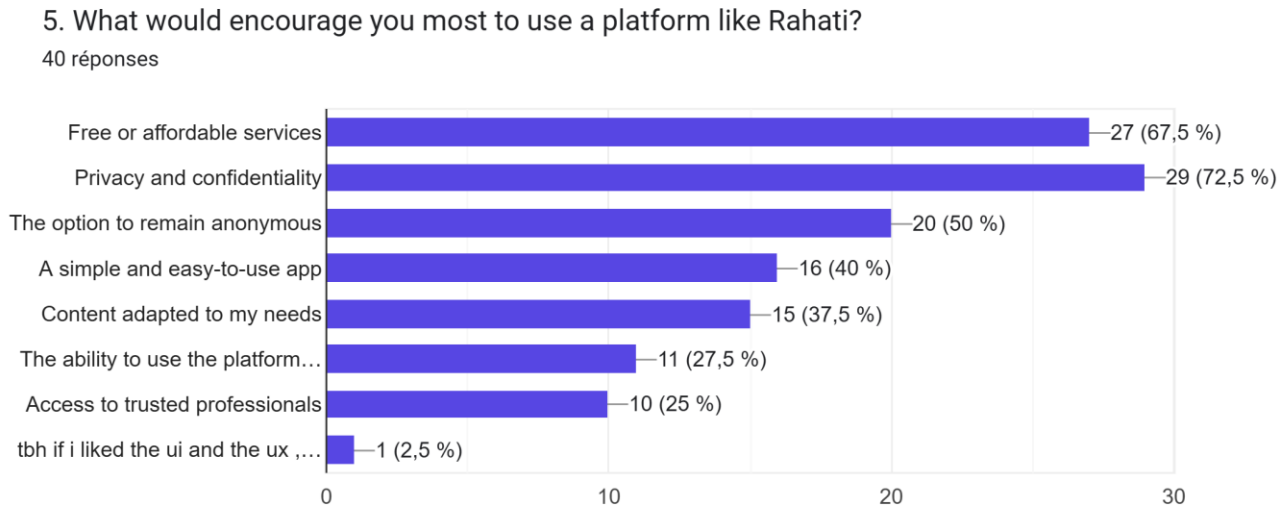
The results suggest that respondents value both human support and self-care tools. Access to a verified female professional was the most frequently selected feature, while practical exercises and personal goals were also highly valued. This indicates that Rahati should not be positioned solely as a tracking application: it should also help users take practical steps and, when needed, find professional support.

Mood tracking over time and weekly progress summaries were each selected by 35% of respondents. These features could help users reflect on their experiences and identify personal patterns. Cycle-related tracking was selected by 25% and should remain optional, respecting differences in users' preferences and needs.

5. Factors That Would Encourage Use and Trust

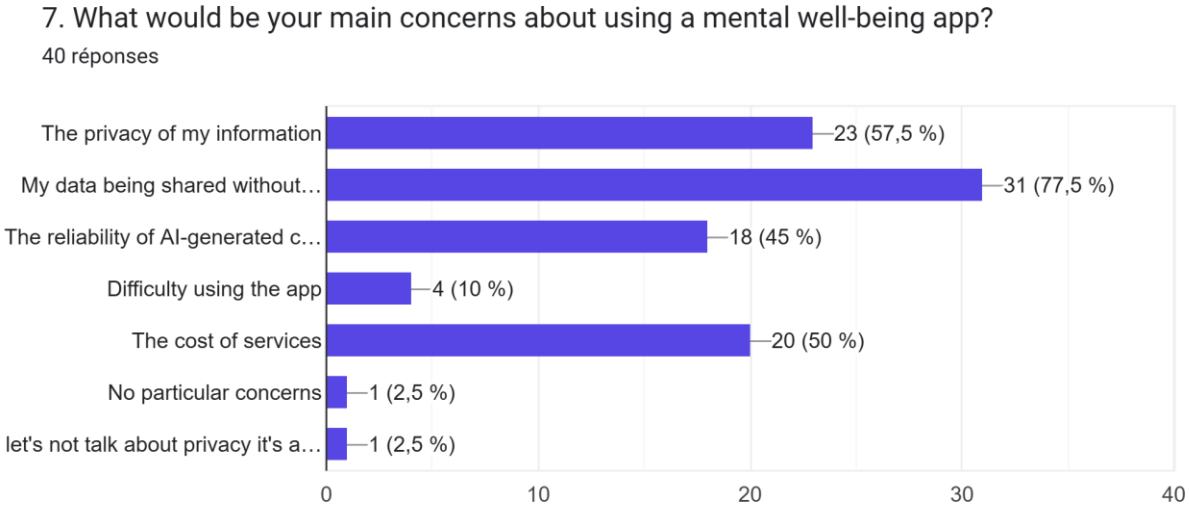
5.1 What would encourage women to use Rahati?

Privacy and confidentiality were the most frequently selected factors, chosen by 29 respondents (72.5%). Free or affordable services followed, selected by 27 respondents (67.5%). The option to remain anonymous was selected by 20 respondents (50%).



5.2 Concerns about using a mental well-being application

The leading concern was the possibility of personal data being shared without permission, selected by 31 respondents (77.5%). The privacy of personal information was selected by 23 respondents (57.5%). The cost of services was a concern for 20 respondents (50%), while 18 respondents (45%) questioned the reliability of AI-generated content.



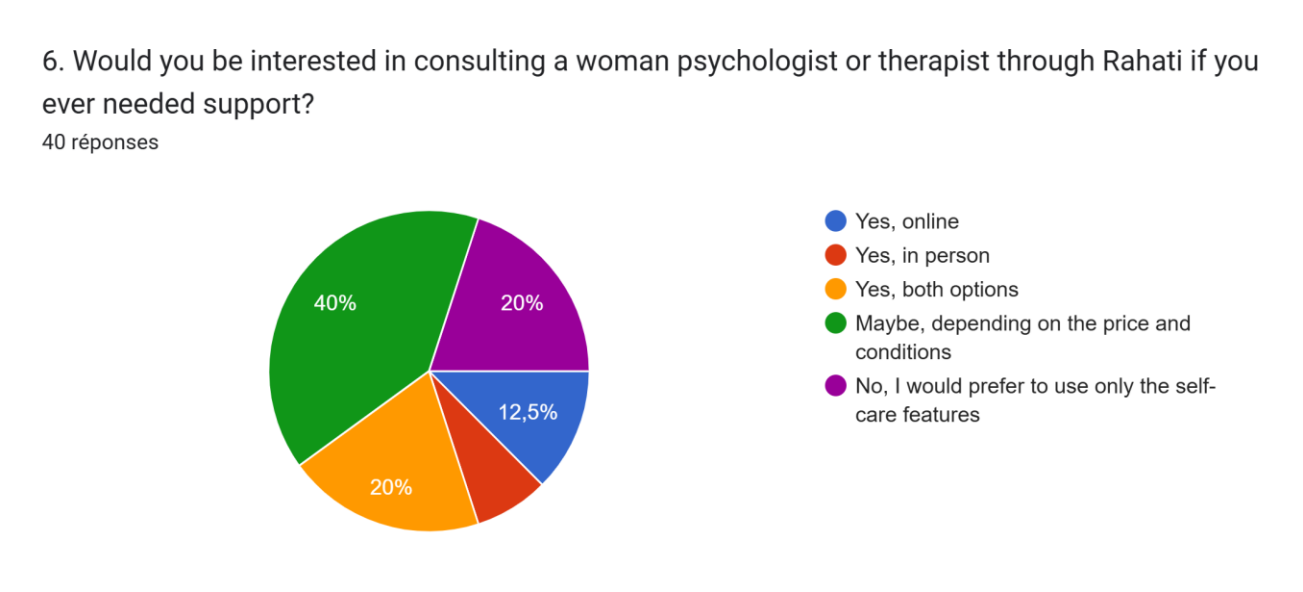
5.3 Interpretation

Trust is a central condition for adoption. The high proportion of respondents concerned about unauthorized data sharing means that privacy should be considered from the design stage, not added as a secondary feature. Rahati should clearly explain what data is collected, why it is collected, how it is used, and what choices users have.

The concern about AI-generated content also has direct implications for the project. Any automated interpretation or suggested advice should be transparent, cautious, and based on reviewed content. AI should not be presented as a substitute for diagnosis or professional care.

6. Interest in Consulting a Woman Psychologist or Therapist

Respondents were asked whether they would be interested in consulting a woman psychologist or therapist through Rahati if they needed support. The chart indicates the following distribution:



The largest group (40%) expressed conditional interest, depending on price and conditions. A further 40% selected one or both consultation formats, while 20% preferred to use self-care features only. This suggests that professional consultation could be offered as an optional pathway, with transparent pricing and different access formats.

Note: the chart labels show 12.5% for online, 20% for both options, 40% for conditional interest, and 20% for self-care only. The remaining 7.5% is inferred from the total and should be checked against the original response export before publication.

7. Open-Ended Suggestions

Nine respondents provided open-ended suggestions for features or improvements. The comments included requests for daily articles or information about mental health, psychology, emotions, and mental health conditions. One respondent suggested a chatbot assistant. Other comments included a request for the possibility of purchasing medicine online, while some respondents had no additional suggestions or were unsure what feature to recommend.

These answers are qualitative and come from a small subset of the 40 respondents. They should be treated as exploratory ideas rather than evidence of the overall prevalence of a particular need. The suggestion

about buying medicine online would require separate regulatory, clinical, and safety assessment and should not be assumed to be a core

8. Preliminary Literature Review and Data Availability

8.1 Research gap identified by the project team

As part of the project, a preliminary literature search was conducted on women's mental health, with particular attention to possible relationships between emotional well-being and menstrual-cycle phases. The sources identified during this initial search appeared limited, especially for research focused on African and Arab women and on how menstrual-cycle-related factors may relate to mood.

This observation should be understood as the outcome of a preliminary search, not as a systematic review proving that very little research exists. A more structured literature review would be needed to establish the size and nature of the research gap.

8.2 The mcPHASES dataset

One relevant dataset identified during the search was mcPHASES (Menstrual Cycle Physiological, Hormonal, and Self-Reported Events and Symptoms). It brings together information on menstrual-cycle phases, hormonal measurements, and self-reported symptoms. The variables shared for the project include phase, LH, estrogen, PdG, flow volume and color, appetite, exercise level, headaches, cramps, breast soreness, fatigue, sleep issues, mood swings, stress, food cravings, indigestion, and bloating.

The dataset is relevant because it makes it possible to explore associations between menstrual-cycle phases, hormonal indicators, and self-reported symptoms such as mood swings. It may therefore support an exploratory prototype examining how reported mood swings vary with cycle phase and other recorded symptoms.

8.3 Limitations of the available indicators

Although mcPHASES includes useful physiological and self-reported information, the available mental and emotional indicators are relatively limited. For example, a mood-swing variable does not by itself provide a complete picture of emotional well-being, psychological distress, stress context, social support, personal circumstances, or the different ways women experience and describe their emotions.

Consequently, the dataset should not be treated as a comprehensive representation of women's mental health. It can support a focused exploratory analysis of cycle-related symptoms and mood swings, but it cannot independently explain the full complexity of women's mental well-being. Its findings should also not be assumed to represent African or Arab women specifically.

8.4 Implications for Rahati's data approach

The project can use mcPHASES as an existing research dataset for an initial exploratory model, while collecting relevant information directly from Rahati users only with informed consent and appropriate privacy safeguards. The application's own tracking data could, over time, help users observe their individual patterns. These two data sources should remain clearly distinguished: mcPHASES is an external research dataset, whereas Rahati's user data reflects the experiences of its own users.

9. Implications for Rahati's Product Design

The following implications are based on the survey findings and preliminary research. They represent design recommendations and potential future developments, rather than features fully implemented in the first Rahati prototype.

Evidence from the questionnaire/review	Potential implication for Rahati
Difficulty understanding emotions (37.5%) and interest in understanding mood changes (50%)	Offer simple mood and energy check-ins, reflection prompts, and visualizations of changes over time.
Stress and anxiety management (57.5%)	Provide accessible stress-management resources and practical exercises.
Daily habits and sleep (52.5%)	Include optional habit and sleep tracking, with simple feedback.
Interest in exercises and personal goals (45%)	Offer small, manageable self-care activities and user-defined goals.
Interest in verified female professionals (47.5%)	Explore a directory or referral pathway to verified professionals.
Privacy and confidentiality (72.5%) and concern about unauthorized sharing (77.5%)	Use privacy-by-design principles, clear consent, data minimization, and transparent data practices.
Interest in cycle-emotion links (27.5%) and availability of mcPHASES	Make menstrual-cycle tracking optional and present any observed associations cautiously.
Concern about AI reliability (45%)	Use reviewed content, explain the limits of automated outputs, and avoid diagnostic claims.

10. Limitations of the Survey

The questionnaire included 40 respondents, and 97.5% were aged 18–24. The sample is therefore small and concentrated in one age group. The results describe the responses of this group and should not be generalized to all women, all age groups, or specific geographic populations without further research.

The questions were self-reported, and most multiple-choice questions allowed several selections. Responses may reflect participants' understanding of the question and their willingness to disclose personal experiences. The open-ended question received nine responses, so its themes are indicative rather than representative.

The survey findings identify reported needs and preferences; they do not establish clinical prevalence, diagnose mental health conditions, or demonstrate causal relationships between menstrual-cycle phases and mood.

11. Conclusion

The questionnaire provides an initial understanding of the needs and expectations of the women who responded. The most prominent findings concern difficulty finding trusted professionals, difficulty understanding emotions, the desire to manage stress and anxiety, and the importance of practical self-care tools. Privacy, confidentiality, affordability, and anonymity emerge as essential conditions for trust and adoption.

The preliminary research review also identifies a limitation in the available data: mcPHASES offers a useful starting point for exploring relationships between menstrual-cycle information and reported mood swings, but its mental health indicators are not sufficient to capture the full complexity of women's emotional and psychological well-being.

Rahati can respond to these findings by combining optional mood and cycle tracking, educational resources, practical self-care activities, progress summaries, and access to trusted professionals. Its design should prioritize privacy and transparency, while any data-driven interpretation should remain cautious, non-diagnostic, and clear about its limitations.

Appendix – Survey Questions Referenced

- Age distribution of respondents.
- What are the main barriers that make it difficult for you to take care of your mental well-being?
- What would you most like to understand or improve about your well-being?
- Which Rahati features would be most useful to you?
- What would encourage you most to use a platform like Rahati?
- Would you be interested in consulting a woman psychologist or therapist through Rahati if you ever needed support?
- What would be your main concerns about using a mental well-being app?
- What feature or improvement would you like to see in Rahati?